

CHAPTER 25 - MOTHERS AND INFANTS (Revised March 2022)

The Mothers and Infants Nurturing Together (MINT) program assists pregnant offenders with making the transition from pregnancy to motherhood. Participants are federal female offenders. Pregnant offenders are eligible to transfer to the MINT Program up to two months prior to their expected delivery date. After delivery, the inmate mother maybe allowed to stay an additional three months to bond with the child, although a minimum of six months is recommended. The mother is then returned to an institution to complete her sentence, if she has not completed her sentence. The intent is to effectively promote maternal bonding and parenting skills in a, "home like environment."

The contractor will provide residents in this program the same general program resources cited in the residential Statement of Work (SOW), except as otherwise specified below:

1. The contractor shall not accept participants in the MINT program unless the RRM has approved them for placement.
2. If the inmate mother is eligible for RRC placement upon completing the MINT program and is within her 10% date of release, she may remain in the MINT program only if she will be supervised in the Judicial District in which the facility is located.
3. Ordinarily, it is expected that the child will be delivered prior to the inmate mother's release from custody. However, in some instances, placement in the MINT program could be used to facilitate pre-natal care for the inmate mother prior to her release and the birth of her infant.
4. The contractor shall have a MINT Coordinator to serve as a liaison between the program and the community. The MINT Coordinator shall do outreach work with the resident, her family, the caretaker of the baby (if different from the resident's family), and social service agencies (i.e., Children's Court, the Welfare Department, foster care).
5. The contractor's facility shall comfortably house women and babies in a secure, safe and healthy environment. The contractor shall house MINT program residents separately and apart from other programs operating out of the residential facility. The contractor will identify specific areas for the MINT program, i.e., dining, recreation, visiting, and sleeping.

6. Ideally, a private room, greater than 60 square feet, should be available for each resident and her baby. This would allow the resident privacy for one-on-one bonding and separation preparation.

7. Residents in this component are to bond with their baby in a "home like environment." However, since the resident is still a federal prisoner their privileges and supervision requirements remain.

a. Residents may only leave the facility when escorted by authorized contract staff.

b. Residents in the MINT program are not ordinarily eligible for passes and/or furloughs; however, the RRM may make an exception to place the baby with relatives or in foster care. The contractor must consult with the RRM in these exceptions.

c. Residents are not eligible to seek or hold gainful employment; therefore, the contractor will not collect subsistence for the MINT program residents.

8. The contractor shall identify local resources to provide needed items (e.g., food, clothing, furnishings, etc.), programs and services for the mother and baby.

The contractor shall contact their local social service program provider to obtain nutritional information, supplemental foods and health screening services for the mother and infant. The Federal Women, Infant, and Children (WIC) program will usually provide this type of service.

The contractor will contact the local Department of Human Resources to learn if the infant is eligible for Aid to Families with Dependent Children (AFDC) and/or Medicaid.

The Contractor will make provisions through state or county clinics for infant immunizations. The BOP will not provide funds for immunizations. The guardian must assume financial responsibility for immunizations. The contractor will provide community resource information that may help the guardian in this need.

9. The contractor shall be responsible for transporting the resident to all necessary outings, medical appointments, counseling, classes (i.e., childbirth, nutrition, and arts and

crafts, if not available at the facility), and to and from the hospital for delivery and release.

10. The contractor shall notify the RRM immediately once the resident is in labor and when being admitted to the hospital. After that, the contractor shall continually update the RRM until the resident delivers the baby. The contractor must provide information such as the resident's location, her condition, complications if any, etc. The contractor shall document this information in the resident's file.

11. The program shall focus on prenatal and postpartum adjustment to include the bonding between the resident and her child and any other family member(s). The contractor will provide counseling services, educational evidence-based recidivism reducing programs and productive activities. Counseling services will include, at a minimum, substance use, mental health, prenatal and postpartum related matters. Educational programs will include, at a minimum, child birthing, prenatal and postpartum self-care, caring for an infant, child development and nutrition, prenatal and postpartum nutrition, positive parenting skills, domestic violence, self-image, substance use, mental health, exercise, and preparing to separate from the child. The resident's family should participate in these sessions when, and as much as possible.

12. The contractor shall provide the appropriate number of meals and food groups recommended for pregnant and lactating residents as deemed required by a certified dietician, nutritionist, or physician. In addition, the contractor will make formula available for the baby to supplement the mother until she can obtain her own supply.

13. The transferring institution is financially responsible for all medical costs incurred by the resident participating in the MINT program. The Bureau of Prisons shall only be financially responsible for the medical expenses of the baby for the first three (3) days after a routine vaginal birth or up to seven (7) days after a cesarean section.

Thereafter, all other expenses will be the responsibility of the person assuming custody of the child. The person assuming custody of the child would be determined prior to the resident entering the RRC and is listed in the file that accompanied the resident from the institution. The MINT Coordinator shall maintain ongoing contact with the infant's guardian to ensure all financial needs are met; however, the MINT Coordinator shall

have community resources in place to supplement where necessary.

The MINT Coordinator shall advise the person assuming custody of the child of any information regarding community public resources that are available to help in the care and welfare of the baby.

14. The contractor shall have an agreement with a licensed OB-GYN to provide prenatal and post-natal care to the MINT program residents. Ideally, the OB-GYN shall be close to the program and the transferring institution. Whenever possible, the transferring institution should use the same OB-GYN.

15. The contractor will complete a report within five working days of the resident's release from the MINT program.

The contractor will fully describe the resident's experience in, and reaction to, the program. It must also summarize counseling made available to the resident and any future progress recommendations.

The contractor will forward the original report to the institution with copies to the RRM and appropriate United States Probation Office.